

Patient Case Framework Instructions

- Use this Patient Case Framework to build out your concept map. For each concept map you create, use the eight guiding questions below. Refer to the Glossary for help.
1. Add your patient case one-liner to the Patient Case box. What are the key elements of the case (history, exam, lab, imaging findings)? Connect them to your Patient Case one-liner using the 'Normal' and 'Abnormal' sticky notes. Which findings are most important for understanding what's going on and why?
 2. What are the important basic science concepts from this week's learning objectives (SLOs)? Add them using the 'Basic Science' and 'Pathophysiology' sticky notes. Be specific.
 3. How do you explain the connections between the basic science concepts/SLOs and the patient's findings? Use lines with arrows to visually show these connections. To make a link, drag from one shape's connector to another. To label the link, double click and briefly describe the mechanism or causal relationship.
 4. Apply your clinical reasoning skills: What are the critical differential diagnoses to be considered for this patient and why? What is the working (most likely) diagnosis and associated illness script? What is your plan for management and why? What are the prognostic considerations? Add sticky notes and links to explain your thought process in answering these questions.
 5. What concepts from Health Systems Science (HSS) influence this case, and how? Add sticky notes and links to explain these concepts and their connections.
 6. What Determinants of Health (DOH) influence this case, and how do they help you shape the patient's condition or access to care? How do they help you understand and interact with the patient? Add DOH icons, lines, and a grey sticky note to illustrate and explain these influences.
 7. How can you connect the material on your concept map to prior coursework? Create sticky notes to explain.
 8. What are your gaps in understanding this week to bring up in your group discussion? Add your questions to the yellow Gaps in Understanding section.

Click for more detailed instructions

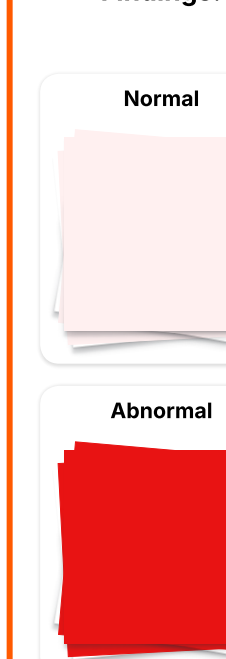
Get help with Lucid

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Toolbox (Drag objects onto your concept map)

Click the notebook icons in the toolbox for more info

Key Case Findings:



Sciences:



Health Systems Science:



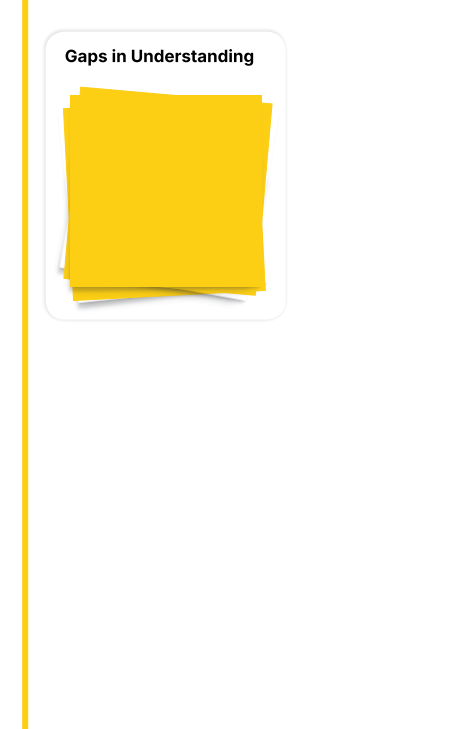
Determinants of Health:



Connections to prior coursework:



Gaps in Understanding



Glossary

Click the notebook icons in the toolbox for more info

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Clinical Science & Diagnostic Reasoning: Synthesizes findings into a cohesive clinical picture that includes:

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- a management plan with rationale. Show how proposed interventions target the pathophysiology and address patient-specific concerns and resource priorities.

Health Systems Science: 6 concepts that impact the patient's presentation and management: Population and public health, Ethics & equity, Epidemiology and biostatistics, Healthcare financing, Patient safety & quality, Care and experience.

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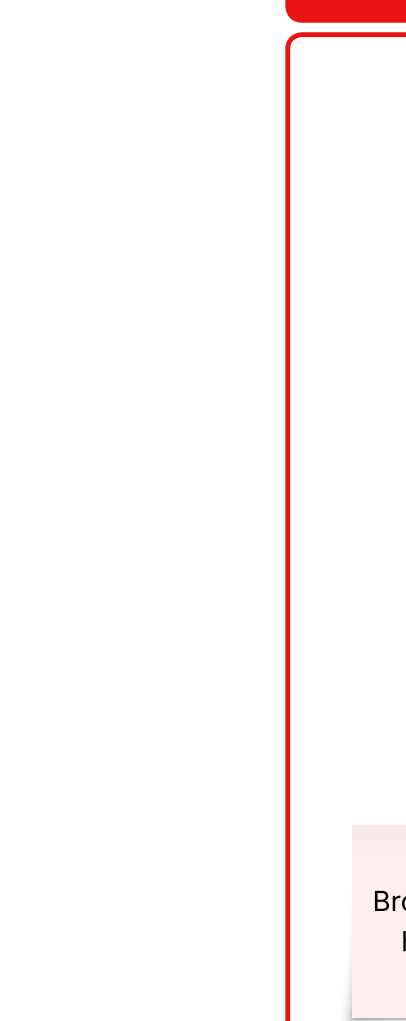
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FMHx:



GENETIC

strong family history of DM2

Risk for Lynch syndrome?

autosomal dominant DNA repair errors are not fixed

associated with CRC, endometrial cancer, and others

microsatellite instability and increased risk of mutations

newly diagnosed PCOS increases risk of endometrial hyperplasia/carcinoma

ovulation of PCOS ovaries produces androgens

unopposed estrogen drives endometrial hyperplasia

advises Julia to report any abnormal bleeding - triggers workup for endometrial cancer

Julia has risk factors for endometrial cancer (obese, PCOS, possible FMHx)

should be well-controlled with OCPs

possible family history of Lynch Syndrome

this is most common in perimenopausal women

ENVIRONMENTAL

access to healthy whole foods, fruit and veggies

reports 2-3 meals, 1-2 snacks per day

less likely given lack of defining HPI features

possible given weight concerns and amenorrhea

BEHAVIORAL

important to build behavioral habits to prevent health's wellness

occasionally goes to the gym but firsts go with classes

anorexia nervosa / bulimia

decreased caloric intake and decreased leptin

suppresses GnRH

denies weight changes, reports overall good mood

normal range 2.5-4kg; 18-20.7 in

normal birth weight (3.2kg) and height (21 in)

maternal hypothyroidism controlled with medication

conclusion last year

TBI can cause hypothyroidism

loss of inhibitory dopaminergic via stalk effect

stalk effect causes blockage of inhibitory dopamine

increases prolactin release

prolactin release suppresses GnRH

suppresses LH and FSH

suppresses ovarian estrogen production

anovulation

subclinical hypothyroidism

initial TSH elevation can signal TSH-mediated prolactin release

mild irregular menses similar to hypothyroidism

++ hair thinning could be early sign

TSH 6 (high), free T4 1.3 (WTN)

subclinical hypothyroidism possibly contributing to oligomenorrhea

monitor TSH/free T4 going forward

to determine whether or not it is contributing to oligomenorrhea

prolactin 11 (WTN)

no prolactinoma

routine medical check-ups needed to monitor prolactin levels due to decreased free T4 for TSH > 10

Medical

Julia Parker is a 19-year-old female who presents to adolescent medicine clinic due to irregular menses; "I haven't had my period."

HPI:

LMP 5 months ago

periods used to occur every 3-10 weeks, last 3-5 days

now unpredictable

cycle length range continues to be 3-12 weeks

now prolonged menses lasting 3-4 weeks

heavy bleeding, large clots

OTC benzoyl peroxide (10%)

antibacterial, follicular desquamation, anti-inflammatory

no changes in weight, diet, exercise

no cold intolerance, hair loss, or constipation

facial hair growth

worsening acne

absent of menstruation > 3 months for previously regular cycles

absence of ovulation at home for previously regular cycles

Julia does not experience any secondary amenorrhea, but we can't rule it out

in a female with LMP 5 months ago, pregnancy and 1st trimester

Does not rule out pregnancy

"all patients pregnant until proven otherwise"

2 romantic partners, both females

reports no sexual intercourse recently

most common cause of secondary amenorrhea

pregnant

combined OCP

opposes endometrial proliferation, thinner lining, less bleeding

estrogen + progestin inhibits FSH/LH

no egg release, no pregnancy

opposes endometrial proliferation, thinner lining, less bleeding

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